



Cyanide

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by the PEDIA Tape.
- Pediatric hypotension is defined as SBP < 70 + (age in years x 2) mmHg

Signs & Symptoms:

- Headache, weakness, vertigo
- Nausea / Vomiting
- Chest Pain / Respiratory Distress
- Tachycardia / Tachypnea
- Severe:
 - Cardiac Arrest
 - Seizures
 - Altered Mental Status / Coma

Differential:

- Acute coronary syndrome
- Stroke / TIA
- PE
- Meningitis / Encephalitis
- Head trauma
- Diabetes
- Acute intoxication
- CO Poisoning

Clinical Management Options

P	P	P	P	P	P	• Scene safety & decontaminate patient as needed
L	L	L	L	L	L	• Oxygen via NRB 15 LPM regardless of SpO₂ reading and passive oxygenation with NC at 25 LPM
1	2	3	4	5	6	• Basic airway maneuvers and management as needed
						• Monitor EtCO₂ as soon as it is practical
						• Vascular access • Hydroxocobalamin
						• Advance airway maneuvers and management as needed

Consult Online Medical Control As Needed

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- Do not begin transport until all contaminated clothing has been removed and patient has been decontaminated and cleared for transport.
- Be alert for exposure related dyspnea/tachypnea without cyanosis, nausea/vomiting, seizures, hyper/hypotension.
- Oxygen via NRB should be applied to all patients as SpO₂ readings are unreliable in presence of cyanide or CO poisoning.
- If smoke inhalation always consider carbon monoxide poisoning and monitor for it if equipped.
- Mix hydroxocobalamin carefully with strict adherence to instructions. Do NOT shake.